



EUROPEAN MEDICINES AGENCY
SCIENCE MEDICINES HEALTH

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Pharmacovigilance Risk Assessment Committee (PRAC)

PRAC recommendations on signals

Adopted at the 31 August-3 September 2026 PRAC meeting

This document provides an overview of the recommendations adopted by the Pharmacovigilance Risk Assessment Committee (PRAC) on the signals discussed during the meeting of 31 August-3 September 2026 (including the signal European Pharmacovigilance Issues Tracking Tool [EPITT]² reference numbers).

PRAC recommendations to provide supplementary information are directly actionable by the concerned marketing authorisation holders (MAHs). PRAC recommendations for regulatory action (e.g. amendment of the product information) are submitted to the Committee for Medicinal Products for Human Use (CHMP) for endorsement when the signal concerns Centrally Authorised Products (CAPs), and to the Co-ordination Group for Mutual Recognition and Decentralised Procedures – Human (CMDh) for information in the case of Nationally Authorised Products (NAPs). Thereafter, MAHs are expected to take action according to the PRAC recommendations.

When appropriate, the PRAC may also recommend the conduct of additional analyses by the Agency or Member States.

MAHs are reminded that in line with Article 16(3) of Regulation No (EU) 726/2004 and Article 23(3) of Directive 2001/83/EC, they shall ensure that their product information is kept up to date with the current scientific knowledge including the conclusions of the assessment and recommendations published on the European Medicines Agency (EMA) website (currently acting as the EU medicines webportal).

For CAPs, at the time of publication, PRAC recommendations for update of product information have been agreed by the CHMP at their plenary meeting (14-17 September 2026) and corresponding variations will be assessed by the CHMP.

For nationally authorised medicinal products, it is the responsibility of the National Competent Authorities (NCAs) of the Member States to oversee that PRAC recommendations on signals are adhered to.

¹ Expected publication date. The actual publication date can be checked on the webpage dedicated to [PRAC recommendations on safety signals](#).

² The relevant EPITT reference number should be used in any communication related to a signal.



Variations for CAPs are handled according to established EMA procedures. MAHs are referred to the available [guidance](#). Variations for NAPs (including via mutual recognition and decentralised procedures) are handled at national level in accordance with the provisions of the Member States.

The timeline recommended by PRAC for submission of variations following signal assessment is applicable to both innovator and generic medicinal products, unless otherwise specified.

For procedural aspects related to the handling of PRAC recommendations on signals (e.g. submission requirements, contact points, etc.) please refer to the [Questions and Answers on signal management](#).

1. Recommendations for update of the product information³

1.1. Oxacillin – Drug reaction with eosinophilia and systemic symptoms (DRESS)

Authorisation procedure	Non-centralised
EPITT No	20223
PRAC Rapporteur	Eva Jirsová (CZ)
Date of adoption	3 September 2026

Recommendation

Having considered the available evidence in EudraVigilance and published literature, including the cumulative review submitted by the Marketing Authorisation Holder (MAH), the PRAC has agreed that the MAHs of oxacillin containing medicinal products should submit a variation within 2 months from the publication of the PRAC recommendation, to amend the product information as described below and taking into account the already existing wording in some nationally authorised products as the text needs to be adapted by MAHs to individual products (new text underlined):

Summary of product characteristics

4.4 Special warnings and precautions for use

The following warning should be added under the paragraph concerning hypersensitivity reactions:

Severe cutaneous adverse reactions (SCARs):

Drug reaction with eosinophilia and systemic symptoms (DRESS), which can be life-threatening or fatal, has been reported in association with oxacillin treatment (see section 4.8). Patients should be advised of the signs and symptoms of this severe cutaneous adverse reaction and should seek medical advice from their physician immediately when observing any indicative signs or symptoms. If signs and symptoms suggestive of DRESS appear, {Invented name} should be withdrawn immediately and an alternative treatment considered (as appropriate). If the patient has developed a severe cutaneous adverse reaction such as DRESS, treatment with oxacillin must not be restarted in this patient at any time.

4.8 Undesirable effects

Under the SOC Skin and subcutaneous tissue disorders with a frequency “Not known”:

Drug reaction with eosinophilia and systemic symptoms (DRESS)

Package leaflet

For products with intravenous/intramuscular route of administration:

2 What you need to know before you use {Invented name}

³ Translations in all official EU languages of the new product information adopted by PRAC are also available to MAHs on the [EMA website](#).

Warnings and precautions

Talk to your doctor before using {Invented name} if you have ever developed a severe skin rash or skin peeling, blistering and/or mouth sores after using this medicine or other related active substances such as other penicillins. This medicine can cause serious skin reaction. Seek medical attention immediately if you notice any of the symptoms related to this serious skin reaction described in section 4.

4 Possible side effects

Seek medical attention immediately if you notice any of the following symptoms of serious skin reactions:

Widespread rash, high body temperature and enlarged lymph nodes (DRESS syndrome or drug hypersensitivity syndrome).

For products with oral administration:

2 What you need to know before you take {Invented name}

Warnings and precautions

Talk to your doctor before taking {Invented name} if you have ever developed a severe skin rash or skin peeling, blistering and/or mouth sores after using this medicine or other related active substances such as other penicillins. This medicine can cause serious skin reactions. Stop taking {Invented name} and seek medical attention immediately if you notice any of the symptoms related to this serious skin reaction described in section 4.

4 Possible side effects

Stop taking {Invented name} and seek medical attention immediately if you notice any of the following symptoms of serious skin reactions:

Widespread rash, high body temperature and enlarged lymph nodes (DRESS syndrome or drug hypersensitivity syndrome).

2. Recommendations for submission of supplementary information

INN	Signal (EPITT No)	PRAC Rapporteur	Action for MAH	MAH
Acitretin	Erectile dysfunction (20310)	Marie Louise Schougaard Christiansen (DK)	Assess in the next PSUR (submission by 20 January 2027)	MAHs of acitretin containing products with an obligation to submit PSURs
Adalimumab; certolizumab pegol; etanercept; golimumab; infliximab	IgA nephropathy (18649)	Karin Bolin (SE)	Supplementary information requested (submission by 11 November 2026)	AbbVie Deutschland GmbH & Co. KG; UCB Pharma SA; Pfizer Europe MA EEIG; Janssen-Cilag International NV
Anakinra	Fulminant hepatitis (20308)	Karin Erneholm (DK)	Supplementary information requested (submission by 3 November 2026)	Swedish Orphan Biovitrum AB
Deucravacitinib	Pemphigoid (20304)	Liana Martirosyan (NL)	Assess in the next PSUR (submission by 17 November 2026)	Bristol-Myers Squibb Pharma EEIG
Fentanyl (transdermal patches)	Accidental exposure (17778)	Liana Martirosyan (NL)	Supplementary information requested (submission by 13 January 2027)	MAHs for fentanyl transdermal patches
Gefitinib	Pulmonary alveolar haemorrhage (20307)	Jenny-Maria Jönsson (SE)	Supplementary information requested (submission by 3 November 2026)	AstraZeneca AB
Idarucizumab	Disseminated intravascular coagulation (20297)	Bianca Mulder (NL)	Assess in the next PSUR (submission by 13 January 2028)	Boehringer Ingelheim International GmbH
Lazertinib	Colitis (20296)	Petar Mas (HR)	Assess in the next PSUR (submission by 29 January 2027)	Janssen Cilag International

3. Other recommendations

INN	Signal (EPITT No)	PRAC Rapporteur	Action for MAH	MAH
Progestogens ⁴	Meningioma (20300)	Bianca Mulder (NL)	No action at this stage	Not applicable

⁴ Chlormadinone/ethinylestradiol, cyproterone acetate/estradiol valerate, cyproterone/ethinylestradiol, dienogest, dienogest/estradiol, dienogest/ethinylestradiol, drospirenone, drospirenone/estetrol, drospirenone/estradiol, drospirenone/ethinylestradiol, dydrogesterone, dydrogesterone/estradiol, estradiol/levonorgestrel, estradiol/norethisterone, estradiol/progesterone, estradiol valerate/norgestrel, ethinylestradiol/gestodene, ethinylestradiol/levonorgestrel, ethinylestradiol/norelgestromin, ethinylestradiol/norethisterone, ethinylestradiol/norgestimate, levonorgestrel, lynestrenol, medroxyprogesterone <100 mg oral formulations, megestrol, nomegestrol acetate/estradiol, norethisterone, progesterone